

SURGICAL PATHOLOGY F

Case Number :



Surgical Pathology Report

Addenda Present

Accession #:

Criteria	Yes	No
Diagnosis Discrepancy		☒
Primary Tumor Site Discrepancy		☒
IPAA Discrepancy		☒
Prior Malignancy History		☒
Qual/Synchronous Primary Noted		☒
Assess (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	Date Reviewed: 8/31/12	

Diagnosis:

A: Urethra, proximal margin, biopsy

- No in situ or invasive carcinoma identified

B: Lymph nodes, right pelvic, dissection

- Five lymph nodes with no malignancy identified (0/5)

C: Lymph nodes, left pelvic, dissection

- Two lymph nodes with no malignancy identified (0/2)

D: Bladder and prostate, cystoprostatectomy

Tumor histologic type: papillary urothelial carcinoma

Tumor histologic grade (WHO classification): high grade

Macroscopic tumor size (greatest dimension): 8.5 cm

Microscopic extent of invasion: tumor invades the lamina propria of the bladder, prostatic ducts and prostatic stroma

Tumor site/focality of tumor: solitary; tumor involves bladder and prostate

Angiolymphatic space invasion: not identified

Associated epithelial lesions: urothelial carcinoma in situ involves the bladder and prostatic ducts bilaterally

Ureters: uninvolved

Histologic assessment of surgical margins:

Ureter, right: negative

100-0-3

Carcinoma, papillary
urothelial

8130/3

Site: bladder, Nos

C67.9

6-6-12 RD

Ureter, left: negative
Urethra: negative
Paravesicular soft tissue: negative
Prostate, right and left: negative

Lymph nodes: Seven lymph nodes with no malignancy identified
(0/7;
specimens A and B)

Other findings: prostatic glandular atrophy; PIN4
immunohistochemical
stain pending on block D7 to rule out prostatic adenocarcinoma
(results will be
reported in an addendum)

AJCC PATHOLOGIC TNM STAGE: pT4a pN0

NOTE: This pathologic stage assessment is based on information
available at the
time of this report, and is subject to change pending clinical
review and
additional information.

Intraoperative Consult Diagnosis:

An intraoperative consultation is requested by Dr. in at
on

FSA1: Urethral margin, biopsy

- Focal urothelial atypia, favor reactive, cannot completely
exclude low grade
dysplasia

- Dr. concurs

Drs. a

Frozen Section Pathologist: , MD

Clinical History:

-year-old with a history of invasive high grade
urothelial carcinoma with
focal micropapillary features and urothelial carcinoma in situ
of the prostate.

Gross Description:

Received are four appropriately labeled containers.

Container A holds a 1.5 x 0.7 x 0.4 cm partially cauterized red/tan soft tissue fragment, FSA1,

Container B is additionally labeled "right pelvic lymph nodes." It holds multiple brown/tan, fatty, lobulated soft tissue fragments which measure 7.2 x 4.5 x 1.7 cm in aggregate. Dissection reveals multiple lymph node candidates, the largest of which is 1.6 x 1.0 x 0.7 cm.

Block summary:

B1 - largest lymph node candidate, bisected
B2 - one lymph node candidate, bisected
B3 - two lymph node candidates
B4 - one lymph node candidate, bisected
Tissue remains in formalin.

Container C is additionally labeled "left pelvic lymph nodes." It holds multiple tan lobulated fatty soft tissue fragments which measure 5.2 x 4.0 x 1.8 cm in aggregate. Dissection reveals two lymph node candidates which measure 1.8 x 1.4 x 0.6 cm and 0.7 x 0.5 x 0.5 cm, respectively.

Block Summary:

C1,C2 - serial sections of largest lymph node candidate
C3 - one lymph node candidate, bisected
Fat remains in formalin.

Container D is additionally labeled "bladder and prostate."

Specimen fixation: formalin

Type of specimen(s) received: radical cystoprostatectomy

Size of overall specimen: 15 cm superior to inferior, 13 cm right to left, 8 cm anterior to posterior

Size of urinary bladder: 8 x 8 x 5 cm

Size of additional organs: The prostate is 4.2 x 3.2 x 3.0 cm

Orientation: Right=blue, left=black, and distal urethral margin=yellow

Tumor size: 8.5 x 7.5 x 3.5 cm

Tumor description: exophytic, papillary friable nodule; Sectioning through the tumor reveals a friable cut surface which appears to be superficial. The underlying muscle wall appears intact as does the perivesicular fat.

Tumor location: The tumor occupies the majority of the urothelium sparing a small portion of the posterior and left wall. The tumor ends inferiorly at the prostatic urethra orifice.

Distance of tumor to margins: Gross tumor is 2.2 cm from the distal prosthetic urethral margin.

Other mucosal lesions: none

Description of other organs or structures: Sectioning through the prostate reveals minimal periurethral nodularity and ill-defined white indurated discoloration in the right posterior/lateral lobe.

Digital photograph taken: not taken

Tissue submitted for special investigation: Tumor is given to

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Block summary:

(Inking: right=blue, left=black, distal urethral margin=yellow)

- D1 - en face right ureter margin
- D2 - en face left ureter margin
- D3 - en face distal urethral margin
- D4-D5 - representative section from right prostate
- D6-D7 - representative sections from left prostate
- D8 - right ureterovesicular junction
- D9 - left ureterovesicular junction

D10 - trigone/bladder neck with underlying seminal vesicle tissue
D11 - section of dome
D12 - right wall
D13 - left wall
D14 - posterior wall
D15 - additional section of left wall
D16 - vas deferens margin of resection
D17 - representative section of blue and black inked deep soft tissue margin

Procedures/Addenda:

Addendum

Addendum

Immunohistochemical stains for PIN4 show an intact basal cell layer, supporting the interpretation of benign prostatic glands.